Patient Name: Eve Vega

DOB: 10/07/1995

Reason for referral: iron deficiency

Patient phone: 530-555-2043

Primary insurance: Aetna

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WakeMed Physician Practices
wakemedphysicians.com

Request for Referral to Gastroenterology

Referral Status *(circle one)*: **STAT** **Urgent** **Routine**

Date:_____/_____/_____

Referring Provider: _____ Practice Name: _____

Practice phone: _____ Practice fax: _____

Patient First Name: _____ Patient Last Name: _____

Date of Birth: _____/_____/_____ Copy of insurance card attached (required)

Best contact phone: _____ Medical Insurance ID #: _____

Patient Address: _____

Reason for referral: office consult screening colonoscopy (without office consult)

Direct scheduling: Please specify procedure requested (referring MD states no office consult needed before procedure):

Procedure requested: _____

Specific question to be addressed in procedure: _____

Diagnosis code (symptoms): _____

_____

Specific Question to be addressed in office consult: _____

_____

☐ Please fax recent/last note, medication list, pertinent labs, copy on insurance card, any GI procedure notes

Refer to: **Next Available** provider or **specific provider** *(circle one provider below)*:

3024 New Bern Ave, Raleigh or 10010 Falls of Neuse Rd, Raleigh

Ph: 919-350-5318 Fax: 919-350-7093

Dr Riaz Chowdhury

Dr Adeyemi Lawal

Dr Juan Herrera

Dr Shannon Scholl

Dr Luis Vazquez-Montesino

110 Kildaire Park Drive, Cary

Ph: 919-235-6440 Fax: 919-235-1389

Dr Dina Ahmad

Dr Mohammad Esmadi

Dr Paul Oh

Dr Sara Ali (after 6/2025)

Please note this form must be completed in its entirety to be processed by WakeMed Gastroenterology.

ClinLab
Clinical Testing Laboratory, Inc.

Clinical Testing Lab of California
55492 Spring Street, Suite 201
Vienna, CA 94078-9932
Phone: 510-555-1212

Specimen Number	Patient ID	Control Number	Account Number	Account Phone Number	Route

Patient Last Name	Account Address

Patient First Name	Patient Middle Name
Jeff	

Patient SS#	Patient Phone	Total Volume

Age (Y/M/D)	Date of Birth	Sex	Fasting
46 v.o.		Male	Yes

Patient Address	Additional Information

Date and Time Collected	Date Entered	Date and Time Reported	Physician Name	NPI	Physician ID

Tests Ordered
CBC, Platelet Ct, and Diff ; Liver Function (AST/ALT) ; Iron/Iron Binding Panel

General Comments

TESTS	RESULT	FLAG	UNITS	REFERENCE INTERVAL	LAB
CBC, Platelet Ct, and Diff					
Hematocrit	40.6		%	40.8 - 51.9	07
Hemoglobin	14.2		g/dL	14.6 - 17.8	07
Red Blood Cell Count	8.4		x10E6/uL	4.69 - 6.07	07
White Blood Cell Count	7.4		x10E3/uL	3.2 - 10.6	07
RDW	13.0		%	10.8 - 14.1	07
MCV	82.6		fL	77.8 - 94.0	07
MCH	27.4		pg	26.5 - 32.6	07
MCHC	34.4		g/dL	32.7 - 36.9	07
Imm.Granulocytes (Absolute)	6.2		x10E3uL	1.3 - 7.0	07
Granulocytes (Percent)	57.2		%	44.0 - 76.0	07
Eosinophils (Absolute)	0.3		x10E3uL	0.0 - 0.4	07
Eosinophils (Percent)	5.2		%	0.0 - 6.0	07
Basophil (Absolute)	0.08		x10E3uL	0.0 - 0.1	07
Basophil (Percent)	1.7		%	0 - 2	07
Monocytes (Absolute)	0.64		x10E3uL	0.2 - 0.7	07
Monocytes (Percent)	6.9		%	4 - 8.9	07
Lymphocytes (Absolute)	2.26		x10E3uL	0.8 - 3.1	07
Lymphocytes (Percent)	37.2		%	15 - 43	07
Platelets (Absolute)	388		x10E3uL	177 - 406	07
Liver Function					
Aspartate aminotransferase (AST)	223	High	U/L	8 - 43	07
Alanine Aminotransferase (ALT)	257	High	U/L	7 - 45	07
Iron and Total Iron Binding					
Serum Iron	2300	Very High	ug/dL	40 - 155	07
Total Iron Binding Capacity	328		ug/dL	250 - 400	07
Transferrin Saturation	72	Very High	%	20 - 50	07

PRELIMINARY INTERPRETATION:

A preliminary diagnosis of Hemochromatosis can be made based on very high Serum Iron and Transferrin Saturation values. Elevated liver enzyme levels are indicative of liver failure which is a common consequence of Hemochromatosis.

There are well established pathogenic genetic variants that are strongly correlated with Hereditary hemochromatosis. The presence of a hereditary disorder has consequences for family members. Genetic testing and counseling should be considered by the patient and family members.